MINDSPRING case review pack (app v45)

For: OpenEvidence clinical review. **Companion document:** "MINDSPRING Clinical Logic v45" (the full rules). **Context:** MINDSPRING is a self-report check-in used by a **licensed psychotherapist** (not a physician) with women aged 40–60. It scores validated screens and sorts follow-up into four urgency tiers: **Today / Within 1 business day / Within 7 days / Next session**. By design it **does not diagnose medical conditions**. When a pattern is more than anxiety or depression alone, it recommends physician referral and adds an optional "physician note". All cases below are synthetic. The outputs are copied exactly from the running app.

Instructions for the reviewer

Step 1. Judge each case blind first. Before reading the app's output for a case, decide from the vignette:

- (a) the menopause stage (STRAW+10);
- (b) what should happen today, within 1 business day, within 7 days, and at the next session;
- (c) any physical red flags that need physician referral.

Step 2. Compare with the app's output. List every disagreement as one row:

| Case | Type | What the app did | What it should do | Evidence (guideline or study) | Severity |

- **Type:** missed flag · unnecessary flag · wrong tier (too urgent or not urgent enough) · wrong stage · wrong score or cut-off · factual error in a physician note · crosses into medical diagnosis · confusing wording for the client.
- **Severity: Critical** (could delay care for suicide risk, violence, or cancer, or cause harm) · **Important** (wrong priority or missing referral) · **Minor** (wording, redundancy).
- If a case is handled correctly, write "No issues".

Step 3. Check consistency across cases. Point out where similar risk is ranked differently between cases without a clear reason. Examples: the suicide-risk tiers in Case E vs. others; the bleeding tiers in Cases F and I; the physical-symptom referrals in Cases D and H.

Step 4. Name the gaps. List up to five things a menopause-informed physician would expect a mood check-in for this age group to catch that it currently misses.

Step 5. Propose edge cases. Write five new short vignettes most likely to break this logic, each with the output you would expect (stage, tier for each flag, referrals). We will run them through the app and send back the results.

Please be specific and cite current guidance where possible: ACOG, The Menopause Society, USPSTF, NICE, VA/DoD, and the Columbia Lighthouse Project. Clinical substance only; no regulatory review is needed.

The cases

Case A

Vignette: 41 years old, regular cycles. Mild worry and tension (GAD-7 items mostly "several days"), mood mostly fine, very few menopause symptoms. Light drinking. No trauma, feels safe at home, partner screen all

"Never".

Scores: PHQ-9 2 · GAD-7 7 · MRS total 3 (No or little), psychological 2, somatic 1, urogenital 0 · AUDIT-C 1 · E-HITS 5

Clinician box:

- Likely stage: Regular cycles: reproductive stage, or very early in the transition
- Medical (hardware) check: No physical warning signals in these answers. Missing answers do not rule them out.
- Therapy focus: PHQ-9 2 (minimal; mood and self-worth items 1, physical and concentration items 1) · GAD-7 7 (mild) · MRS psychological 2 (mild)

Flags (in order):

- None

Case B

Vignette: 44 years old, cycle length now varies by more than 7 days. History of prior depression and PMDD. Low mood, sleep problems "more than half the days", severe sleep problems on the MRS, hot flushes moderate. Mood changes started around the time cycles changed. No thoughts of death.

Scores: PHQ-9 12 · GAD-7 6 · MRS total 16 (Moderate), psychological 7, somatic 7, urogenital 2 · AUDIT-C 0

Clinician box:

- Likely stage: Early menopause transition (perimenopause): cycle length varies by 7+ days
- Medical (hardware) check: No physical warning signals in these answers. Missing answers do not rule them out.
- Next session: 1 item: Perimenopause with a history of prior depression, pmdd
- Therapy focus: PHQ-9 12 (moderate; mood and self-worth items 5, physical and concentration items 7) · GAD-7 6 (mild) · MRS psychological 7 (severe)

Flags (in order):

- **[Next session · clinical]** Perimenopause with a history of prior depression, pmdd: the group at highest risk for perimenopausal depression. Monitor mood through the transition, even if current scores are low.

Case C

Vignette: 47 years old with a hormonal IUD (periods hidden). Strong worry and restlessness (GAD-7 high). Answers Yes to the OCD gate: obsessing and checking high on the OCI-R, other subscales low. History of anxiety disorder.

Scores: PHQ-9 6 · GAD-7 15 · MRS total 12 (Moderate), psychological 7, somatic 5, urogenital 0 · OCI-R 20

Clinician box:

- Likely stage: Can’t be determined from periods (hormonal IUD; periods reported as hidden by contraception, surgery, or medication); symptoms, age, and if needed blood tests guide staging

- Medical (hardware) check: No physical warning signals in these answers. Missing answers do not rule them out.
- Next session: 2 items: MRS psychological vs PHQ-9 mismatch; OCI-R below cut-off
- Therapy focus: PHQ-9 6 (mild; mood and self-worth items 3, physical and concentration items 3) · GAD-7 15 (severe) · MRS psychological 7 (severe)

Flags (in order):

- **[Next session · clinical]** MRS psychological 7/16 (severe) but PHQ-9 6 (below 10). Mood symptoms may be under-reported on the PHQ-9 or concentrated in irritability, anxiety, and exhaustion; clarify in session.
- **[Next session · clinical]** OCI-R 20/72, below the cut-off of 21. Subscales: Obsessing 9/12, Checking 8/12, Ordering 3/12, Washing 0/12, Neutralizing 0/12, Hoarding 0/12. The OCI-R covers mental rituals poorly (reviewing conversations, reassurance seeking, mental checking); ask about these directly. If the distress centers on replaying conversations afterward and fearing you did something wrong (post-event rumination), assess social anxiety as well as OCD.

Case D

Vignette: 50 years old, no period for 3–11 months. Very severe hot flushes, severe sleep problems, severe vaginal dryness, moderate sexual problems. Not on hormone therapy. Drinks 4+ times a week, 1–2 drinks, 6+ drinks monthly. Mild mood symptoms.

Scores: PHQ-9 7 · GAD-7 7 · MRS total 22 (Severe), psychological 6, somatic 10, urogenital 6 · AUDIT-C 6

Clinician box:

- Likely stage: Late menopause transition (perimenopause): 3–11 months without a period
- Medical (hardware) check: Likely physical contribution, 2 signals: Severe hot flushes, untreated; Vaginal and urinary symptoms. Refer to physician; this is more than anxiety or depression alone.
- Within 7 days: 3 items: Severe hot flushes, untreated; Vaginal and urinary symptoms; AUDIT-C 6/12 with severe sleep problems
- Therapy focus: PHQ-9 7 (mild; mood and self-worth items 2, physical and concentration items 5) · GAD-7 7 (mild) · MRS psychological 6 (moderate)

Flags (in order):

- **[Within 7 days · medical]** Hot flushes and sweating rated very severe with no current hormone therapy: a physical symptom that also drives sleep and mood; discuss treatment options with the physician. CBT and clinical hypnosis also reduce hot-flush bother (Menopause Society 2023). *Physician note: Hormone therapy is the most effective treatment for many women; non-hormonal options include fezolinetant, some SSRIs/SNRIs, and gabapentin.*
- **[Within 7 days · medical]** Vaginal dryness (severe) with urogenital symptoms (MRS urogenital 6/12): physical symptoms beyond mood; include in the physician referral. Ask about pain with sex and urinary symptoms. *Physician note: Fits genitourinary syndrome of menopause (GSM): low-dose vaginal estrogen is first-line (AUA/SUFU/AUGS 2025), with moisturizers and lubricants.*
- **[Within 7 days · clinical]** AUDIT-C 6/12, at or above the usual cut-off for women (3). AUDIT-C detects risky drinking better than it diagnoses alcohol use disorder. Raised priority: alcohol worsens severe sleep problems, so address it alongside them.

Case E

Vignette: 52 years old, 12+ months without a period. PHQ-9 total 19 with item 9 "more than half the days". C-SSRS: yes to wish to be dead and to active suicidal thoughts; no method, intent, plan, or past behavior; no self-harm. Prior depression.

Scores: PHQ-9 19 · GAD-7 10 · MRS total 20 (Severe), psychological 11, somatic 6, urogenital 3

Clinician box:

- Likely stage: Postmenopause: 12+ months without a period
- Medical (hardware) check: No physical warning signals in these answers. Missing answers do not rule them out.
- Today: 3 items: PHQ-9 item 9 positive: item 9 more than half the days; Why urgent despite low screen; Suicide-risk follow-up (PHQ-9 item 9 positive)
- Therapy focus: PHQ-9 19 (moderately severe; mood and self-worth items 11, physical and concentration items 8) · GAD-7 10 (moderate) · MRS psychological 11 (severe)

Flags (in order):

- **[Today · urgent]** Thoughts of death or self-harm were reported (PHQ-9 item 9: more than half the days). This needs a prompt, direct assessment by a qualified professional. If you might act on these thoughts or cannot stay safe, call 911 or go to an emergency department; in the U.S., call or text 988.
- **[Today · clinical]** Rated urgent despite a low C-SSRS screen because of: item 9 more than half the days. Same-day contact and a full risk assessment.
- **[Today · clinical]** C-SSRS screener: low risk (yes to items 1, 2). A screener result is not a risk assessment; a full clinical evaluation is needed.

Case F

Vignette: 55 years old, both ovaries removed at 46, breast cancer history, takes tamoxifen. Reports bleeding between periods. Not on hormone therapy. Hot flushes severe, vaginal dryness moderate. Mood mild.

Scores: PHQ-9 6 · GAD-7 3 · MRS total 16 (Moderate), psychological 5, somatic 7, urogenital 4

Clinician box:

- Likely stage: Surgical menopause (both ovaries removed): postmenopausal from the date of surgery
- Medical (hardware) check: Likely physical contribution, 5 signals: Abnormal bleeding on tamoxifen; Hormone-affecting treatments; Takes tamoxifen; Severe hot flushes, untreated; Vaginal and urinary symptoms. Refer to physician; this is more than anxiety or depression alone.
- Within 1 business day: 1 item: Abnormal bleeding on tamoxifen
- Within 7 days: 4 items: Hormone-affecting treatments; Takes tamoxifen; Severe hot flushes, untreated; Vaginal and urinary symptoms
- Therapy focus: PHQ-9 6 (mild; mood and self-worth items 3, physical and concentration items 3) · GAD-7 3 (minimal) · MRS psychological 5 (moderate)

Flags (in order):

- **[Within 1 business day · medical]** Bleeding (bleeding between periods) while taking tamoxifen: needs prompt medical evaluation. *Physician note: Gynecologic evaluation (pelvic ultrasound, often with endometrial sampling). Tamoxifen raises endometrial cancer risk.*
- **[Within 7 days · medical]** Treatments or history that affect hormones (Both ovaries removed; Breast, endometrial, or other hormone-sensitive cancer): these change how menopause and mood symptoms are read; coordinate with the medical clinician. *Physician note: Affects menopause staging and which treatments are suitable.*
- **[Within 7 days · medical]** Takes tamoxifen: this affects how menopause symptoms are read and which medicines are suitable. *Physician note: When choosing an antidepressant or hot-flash treatment, avoid strong CYP2D6 inhibitors (paroxetine, fluoxetine, bupropion), which lower endoxifen levels; duloxetine is a moderate inhibitor. Little interaction: venlafaxine, escitalopram, citalopram.*
- **[Within 7 days · medical]** Hot flushes and sweating rated severe with no current hormone therapy: a physical symptom that also drives sleep and mood; discuss treatment options with the physician. CBT and clinical hypnosis also reduce hot-flush bother (Menopause Society 2023). *Physician note: With a hormone-sensitive cancer history, hormone therapy is usually avoided; non-hormonal options include fezolinetant, some SSRIs/SNRIs, and gabapentin (on tamoxifen, avoid paroxetine and fluoxetine).*
- **[Within 7 days · medical]** Vaginal dryness (moderate) with urogenital symptoms (MRS urogenital 4/12): physical symptoms beyond mood; include in the physician referral. Ask about pain with sex and urinary symptoms. *Physician note: Fits genitourinary syndrome of menopause (GSM): low-dose vaginal estrogen is first-line (AUA/SUFU/AUGS 2025), with moisturizers and lubricants; start only after the reported bleeding has been evaluated.*

Case G

Vignette: 57 years old, postmenopausal. Trauma gate Yes; PC-PTSD-5 4 of 5; PCL-5 46/80. Childhood adversity Yes. Partner screen: physically hurt "sometimes", insulted and threatened "fairly often", screamed at "sometimes", forced sex "never".

Scores: PHQ-9 10 · GAD-7 13 · MRS total 17 (Severe), psychological 9, somatic 6, urogenital 2 · E-HITS 15

Clinician box:

- Likely stage: Postmenopause: 12+ months without a period
- Medical (hardware) check: No physical warning signals in these answers. Missing answers do not rule them out.
- Within 1 business day: 3 items: PCL-5 item 16 (taking risks or doing things that could cause harm) rated...; [private] Relationship safety screen (E-HITS) positive; [private] Physical harm by a partner (sometimes) reported, regardless of E-HITS total
- Next session: 3 items: PC-PTSD-5 positive; PCL-5 total 46 of 80 (at or above the provisional PTSD cut-off of 31–33); Childhood adversity
- Therapy focus: PHQ-9 10 (moderate; mood and self-worth items 5, physical and concentration items 5) · GAD-7 13 (moderate) · PCL-5 46/80 · MRS psychological 9 (severe)

Flags (in order):

- **[Within 1 business day · clinical]** PCL-5 item 16 (taking risks or doing things that could cause harm) rated moderately: ask directly about risky or self-harming behavior.
- **[Within 1 business day · clinical, private]** Relationship safety screen (E-HITS) positive: 15/25 (cut-off 7 or more). Discuss privately; safety planning may be needed. Check whether any reporting rules apply.

- **[Within 1 business day · clinical, private]** Physical harm by a partner (sometimes) reported, regardless of E-HITS total. Discuss privately and check whether any reporting rules apply.
- **[Next session · clinical]** PC-PTSD-5 positive: 4 of 5 (cut-off 3, used here for women; the VA general cut-off is 4). Probable PTSD; a full trauma assessment is warranted.
- **[Next session · clinical]** PCL-5 total 46 of 80 (at or above the provisional PTSD cut-off of 31–33). Clusters: intrusions 13/20, avoidance 5/8, negative thoughts and mood 13/28, arousal and reactivity 15/24. Items 15, 19, and 20 (irritability, concentration, sleep) overlap with menopause symptoms. Repeat to track change.
- **[Next session · clinical]** Childhood experiences that still affect the client: explore childhood adversity in session (the full ACE questionnaire can be used there).

Case H

Vignette: 59 years old, postmenopausal. Bowel gate Yes: symptoms started within the past year; bloating or bigger belly and feeling full quickly on most days; weight loss she was not trying for; stools mostly normal; colonoscopy within 10 years. Bladder symptoms moderate. PHQ-9 11.

Scores: PHQ-9 11 · GAD-7 3 · MRS total 14 (Moderate), psychological 6, somatic 4, urogenital 4

Clinician box:

- Likely stage: Postmenopause: 12+ months without a period
- Medical (hardware) check: Likely physical contribution, 2 signals: Bowel warning signs; New bloating, fullness, or pelvic pain. Refer to physician; this is more than anxiety or depression alone.
- Within 1 business day: 2 items: Bowel warning signs; New bloating, fullness, or pelvic pain
- Next session: 2 items: Ask about stool leakage; Gut symptoms
- Therapy focus: PHQ-9 11 (moderate; mood and self-worth items 4, physical and concentration items 7) · GAD-7 3 (minimal) · MRS psychological 6 (moderate)

Flags (in order):

- **[Within 1 business day · medical]** Bowel warning signs (unintended weight loss; new bowel symptoms after age 50): needs medical evaluation before these symptoms are put down to IBS, stress, or menopause. Blood in stool, weight loss, or anemia need prompt review.
- **[Within 1 business day · medical]** Symptoms on most days (bloating or a bigger belly; early fullness or loss of appetite; plus bladder symptoms rated moderate), starting within the past year: a new physical pattern that needs prompt medical review. It is usually not serious, but it should be checked. *Physician note: Overlaps with early ovarian cancer symptoms (new, more than 12 days a month): pelvic exam, ultrasound, and CA-125 as the physician decides.*
- **[Next session · clinical]** Bladder or vaginal symptoms are moderate or worse; ask directly about straining and stool leakage, which people rarely volunteer.
- **[Next session · clinical]** Stomach or bowel symptoms reported (normal stools). Medical evaluation of the flags above comes first. If IBS is confirmed, IBS-specific CBT and gut-directed hypnotherapy are effective (roughly 60–70% respond).

Case I

Vignette: 60 years old, on menopausal hormone therapy (periods hidden). Ticked "bleeding after 12 months without a period". Marked pregnancy "possible: unsure". Few mood symptoms.

Scores: PHQ-9 2 · GAD-7 1 · MRS total 8 (Mild), psychological 1, somatic 4, urogenital 3

Clinician box:

- Likely stage: Likely postmenopause, based on age 60 (periods can't be used: current menopausal hormone therapy; periods reported as hidden by contraception, surgery, or medication)
- Medical (hardware) check: Likely physical contribution, 1 signal: Bleeding after menopause. Refer to physician; this is more than anxiety or depression alone.
- Within 1 business day: 1 item: Bleeding after menopause
- Next session: 2 items: Currently using menopausal hormone therapy; Pregnancy answer doesn't fit stage or age
- Therapy focus: PHQ-9 2 (minimal; mood and self-worth items 0, physical and concentration items 2) · GAD-7 1 (minimal) · MRS psychological 1 (no or little)

Flags (in order):

- **[Within 1 business day · medical]** Bleeding after 12 months without a period always needs prompt medical evaluation. Contact a medical clinician soon; do not wait for a routine visit. If you use hormone therapy, tell your prescriber: scheduled withdrawal bleeds can be expected, but unscheduled bleeding still needs evaluation. *Physician note: Postmenopausal bleeding: 1–14% have endometrial cancer (ACOG 2018); evaluation is by transvaginal ultrasound or endometrial sampling.*
- **[Next session · clinical]** Currently using menopausal hormone therapy: low hot-flush or MRS scores may reflect treatment working, not absence of symptoms.
- **[Next session · clinical]** Check answers: pregnancy marked possible or unsure, but the answers indicate postmenopause. Pregnancy is very unlikely; clarify what the client meant.

Case J

Vignette: 45 years old, cycles vary. Attention problems since childhood, never diagnosed; memory worsening. ASRS Part A all six in the shaded range. Eating gate Yes: SCOFF 2 of 5, loss-of-control eating weekly with distress, no compensatory behaviors. Body changes affect self-image "a great deal". PHQ-9 10.

Scores: PHQ-9 10 · GAD-7 8 · MRS total 12 (Moderate), psychological 7, somatic 4, urogenital 1 · SCOFF 2

Clinician box:

- Likely stage: Early menopause transition (perimenopause): cycle length varies by 7+ days
- Medical (hardware) check: Likely physical contribution, 2 signals: Worsening memory or thinking; Weight-loss treatment caution. Refer to physician; this is more than anxiety or depression alone.
- Within 7 days: 4 items: Worsening memory or thinking; Weight-loss treatment caution; SCOFF positive (2/5); Binge eating (weekly or more)
- Next session: 2 items: Attention problems since childhood, never diagnosed; ASRS Part A positive
- Therapy focus: PHQ-9 10 (moderate; mood and self-worth items 4, physical and concentration items 6) · GAD-7 8 (mild) · MRS psychological 7 (severe)

Flags (in order):

- **[Within 7 days · medical]** Memory or thinking changes that are worsening, impairing, or noticed by others: get a medical assessment; do not assume menopause or mood is the cause.

- **[Within 7 days · medical]** Possible eating disorder: the physician should know before restrictive diets, weight-loss medication, or surgery are recommended, because these can reactivate disordered eating.
- **[Within 7 days · clinical]** SCOFF 2 of 5 (positive at 2 or more): an eating disorder is likely. Confirm with a clinical interview against DSM-5 criteria; the EDE-Q can measure severity. The SCOFF was built for anorexia and bulimia and has not been validated specifically in women over 50, so read it as a prompt for interview.
- **[Within 7 days · clinical]** Loss-of-control overeating reported (about once a week or more, with distress). This fits the frequency and distress criteria for binge eating disorder if confirmed; Ask about eating in secret, eating when not hungry, and guilt afterward; a dedicated screener such as the BEDS-7 can be given in session.
- **[Next session · clinical]** Attention problems since childhood, never diagnosed: consider a full ADHD evaluation. A midlife screening score alone can't separate ADHD from menopause-related brain fog.
- **[Next session · clinical]** ASRS Part A positive: 6 of 6 in the shaded range, highly consistent with adult ADHD; further evaluation is warranted. Childhood onset: reported. Perimenopause and poor sleep can raise these scores, so the evaluation should confirm childhood onset.

Case K

Vignette: 48 years old, regular cycles, no symptoms of note (control case). Answers No to every gate.

Scores: PHQ-9 0 · GAD-7 0 · MRS total 2 (No or little), psychological 0, somatic 2, urogenital 0 · AUDIT-C 0

Clinician box:

- Likely stage: Regular cycles: reproductive stage, or very early in the transition
- Medical (hardware) check: No physical warning signals in these answers. Missing answers do not rule them out.
- Therapy focus: PHQ-9 0 (minimal; mood and self-worth items 0, physical and concentration items 0) · GAD-7 0 (minimal) · MRS psychological 0 (no or little)

Flags (in order):

- None

Case L

Vignette: 53 years old, last period about 5 months ago. Feels unsafe at home: Yes. Partner questions: prefers to discuss privately. Trauma: prefers to discuss in session. GAD-7 15, PHQ-9 12, MRS psychological severe.

Scores: PHQ-9 12 · GAD-7 15 · MRS total 18 (Severe), psychological 10, somatic 6, urogenital 2

Clinician box:

- Likely stage: Late menopause transition (perimenopause): last period 161 days ago (60 days to 12 months)
- Medical (hardware) check: No physical warning signals in these answers. Missing answers do not rule them out.
- Today: 1 item: [private] Feels unsafe at home
- Next session: 2 items: Client prefers to discuss trauma history in session; [private] Home and relationship safety: discuss privately

- Therapy focus: PHQ-9 12 (moderate; mood and self-worth items 5, physical and concentration items 7) · GAD-7 15 (severe) · MRS psychological 10 (severe)

Flags (in order):

- **[Today · urgent, private]** You said you feel unsafe at home. If you are in danger, call 911. For confidential support, call the National Domestic Violence Hotline at 1-800-799-7233 or text START to 88788.
- **[Next session · clinical]** Client prefers to discuss trauma history in session.
- **[Next session · clinical, private]** Client prefers to discuss home and relationship safety privately. Ask in session, somewhere the client can speak freely.